

IN, Milo DOB: 10/24/2023 (2 yo M) Acc No. 194924 DOS: 01/07/2026



In, Milo

2Y 2M old Male, DOB: 10/24/2023

Account Number: 194924

27156 Allegretto Dr, Spring, TX-77386-4914

Home: 334-492-1546

Guarantor: In, Sokkay Insurance: Holista (Aetna-Therapy Providers) Payer ID: ATHAL

Referring: Michelle Suhendra

Appointment Facility: Houston Center for Pediatric Therapy

01/07/2026

Appointment Provider: Kandis Dennis, PT Resource: Smith, Savanna
Supervising Provider: Kandis Dennis, PT

Reason for Appointment

1. Physical Therapy

Examination

Physical Examination:

Recommended New Goals

The following goals will allow Milo to develop the necessary range of motion, strength, coordination, balance, and body awareness to advance toward independent age-appropriate mobility and independent age-appropriate gross motor skills.

1. Short Term Goal: Milo will maintain quadruped position for 30 sec with CGA or less 2/3 trials in order to demonstrate improved strength and prepare for crawling on hands and knees.

Baseline: Milo is able to assume quadruped position but only maintains briefly due to poor tolerance for position, requiring blocking to maintain due to resistance.

2. Short Term Goal: Milo will crawl forward 5 feet on hand and knees using cross-lateral pattern demonstrating proper lower extremity alignment 2 out of 3 trials.

Baseline: Limited progress toward crawling on hands and knees due to poor tolerance for quadruped position and preference to collapse into prone and roll toward LEFT side during attempts at facilitating creeping forward. Milo demonstrates a strong preference for scooting forward on his bottom using his RLE and LUE.

3. Short Term Goal: Milo will transition from sitting to tall kneeling at support surface demonstrating proper lower extremity alignment and BOS 3 out of 4 trials.

Baseline: Milo continues to perform with excessive BLE external rotation with preference to transition from ring sitting to tall kneeling, resulting in wide BOS. Patient requires min-mod A to perform from side-sitting due to resistance.

4. Short Term Goal: Milo will maintain tall kneeling without UE support with min A and appropriate alignment and BOS for 10 sec 3/4 trials in order to demonstrate improved core and hip strength for age-appropriate transitions.

Baseline: Tactile cues to perform with narrow BOS and neutral LE alignment due to preference for BLE external rotation and abduction. Once positioned, Milo maintains briefly at support surface but not

Progress Note: Kandis Dennis, PT 01/07/2026

mid-floor without UE support.

5. Short Term Goal: Milo will pull to stand via half kneeling with each foot leading 3/4 trials with min A in order to demonstrate symmetrical LE strength and prepare for standing and cruising.
Baseline: Strong preference to assume half kneel with RIGHT foot leading. Min-mod A to consistently transition into standing

6. Short Term Goal: Milo will perform trunk rotation while standing at support surface with 1 UE support to reach for toy to each side 3/4 trials without LOB with CGA.

Baseline: Milo is able to maintain standing with 2 UE support on support surface with SBA, consistently. Patient is not yet able to perform without UE support or support provided at trunk. Milo attempts to lower to the floor when therapist facilitates reaching from UE support or requires min-mod A at trunk to prevent LOB.

7. Short Term Goal: Milo will transition from standing to sitting using stable object for support demonstrating proper lower extremity alignment 3 out of 4 trials with CGA and without LOB.

Baseline: Milo has demonstrated good progress with controlled lowering but does not always maintained UE support on support surface. Milo also demonstrates tendency to extend LEFT knee, requiring occasional tactile cueing and blocking to maintain L foot contact with the floor.

8. Short Term Goal: Milo will sidestep 4 steps in each direction along stable object demonstrating proper lower extremity alignment 3 out of 4 trials with min A.

Baseline: mod-max A to perform

9. Short Term Goal: Milo will ambulate forward 5 ft with push-toy with min A 3/4 trials without LOB in order to prepare for ambulation.

Baseline: Mod A to perform 2 steps forward before attempting to lower to the ground.

10. Short Term Goal: Milo will maintain standing mid-floor without UE support with SBA for 5 sec 3/4 trials in order to demonstrate improved balance and stability in standing.

Baseline: Mod A

11. Short Term Goal: Milo will ambulate 10 feet with shoulder width base of support with 2 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: Milo does not attempt stepping with 2 hand-held support. Milo has demonstrated up to 2 steps with push-toy with mod A at trunk.

12. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Long Term Goals have been established, as discharge criteria for the dates requested for this episode of care:

1. Milo will pull to stand via half kneeling with each foot leading 3/4 trials IND in order to demonstrate symmetrical LE strength and prepare for standing and cruising.

Baseline: See short term goal

2. Milo will cruise along support surface 6 steps to each side 3/4 trials IND without LOB in order to demonstrate symmetrical weight-bearing and prepare for IND stepping.

Baseline: See short term goal

3. Milo will ambulate 10 feet with shoulder width base of support with 1 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: See short term goal

4. Milo will perform squat to/from stand 3/4 trials with min A with symmetrical weight-bearing in order to safely retrieve objects from the floor.

Baseline: Mod A. Demonstrates increased tendency to extend left knee.

5. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Clinical Assessment Summary

Doctor's Name: Dr. Michelle Suhendra, M.D.

Phone: 832-539-7532

Fax: 832-336-3809

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Physical Examination

Physical Therapy:

Subjective

Time In: 03:00 pm

Time Out: 03:55 pm

Present for session: Patient transitioned well from parents in waiting area to treatment area with PT.

Patient participation: Patient participated well overall. Patient continues to demonstrate decreased tolerance for quadruped position and strong preference for scooting on his bottom with left weight shift, using RLE to pull himself forward. However, slight improved tolerance to therapist facilitating crawling on hands and knees briefly this session.

Parent reported: Patient has been climbing onto furniture and standing a lot recently.

Home exercise program: Parents to continue HEP including activities from Position to Play for sitting and prone and cervical strengthening exercises and tummy time. Quadruped with or without support of roll or support and reaching with UEs, tall kneeling supported on low cushion with assist to hold position, standing supported by stable object, half kneeling with assist as needed (emphasis on LLE leading due to preference for RLE).

Note:

Evaluated for SMOs at Align on 12/09.

Objective

(97530) Therapeutic Activities to progressively improve functional performance. Direct Treatment Time: 30 minutes

Transitions:

*Moderate assist to assume quadruped position from prone and sitting due to poor tolerance for quadruped position.

* Quadruped to tall kneel with CGA x2

*Transitioned from sitting to tall kneeling at 16" surface multiple reps with SBA but with hip ER due to transitioning from ring-sitting. Performed from side-sitting x4 from each side with min-mod A

*Transition from prone to sitting: preference for performing from his left side with tactile cues and blocking to perform from his right side

*Transition from sitting to prone: min-mod A to position BLE due to preference for scooting on bottom.

* Sit to/from stand on therapist's leg: tactile cues or blocking to maintain feet in contact with floor due to extending L knee. Min A to stand. Min-mod A to facilitate bilateral knee flexion to return to sitting. Mass practice

* Pull to stand via half kneel: assumed half kneel with R foot leading multiple reps with SBA-CGA. Blocking and tactile cues to assume with LEFT foot leading. mod A to pull up with emphasis on L foot leading x8

NPT* Pull to sit from supine on floor mat x2: head lag demonstrated during first 50%

NPT* Straddling peanut ball while reaching overhead to pull squigz from mirror x1 min, stabilization provided at pelvis

NPT* Sitting on therapy ball with facilitation of supine to sit with mod A x3

NPT* Side-lying to sit on therapy ball x1 from each side with mod A

Mobility:

NPT* Rolling supine to prone: Rolling toward the left and right independently.

NPT* Rolling prone to supine: preference for rolling toward left side demonstrated, requiring blocking to perform toward right side x1

NPT* Commando crawling forward: 2 ft x1 with tactile cues.

Resistance demonstrated to additional attempts due to preference for scooting

* Cruising with 2 UE support at support surface: 2-4 steps to each side with mod A and intermittent max A when resisting x3

NPT* Forward stepping with push-toy: 2-4 steps forward with increased trunk flexion toward push-toy. Performed x2 with mod A at pelvis

NPT* Scooting forward on bottom with L arm wrapped against trunk with fabrifoam to promote using each LE due to strong preference to

perform with weight-bearing through LUE and scooting with RLE.
Performed 3 ft forward with max A for weight-shifting to each side x6
* Scooting with blocking of LUE via 2 HHA to promote using BLE to scoot forward due to strong RLE preference
NPT* Commando crawling backward down ramp x3 with SBA
NPT* Commando crawling up ramp: attempted to facilitate but patient demonstrated resistance with preference to transition back to sitting
* Crawling on hands and knees:
- on floor mat forward 1 ft with mod A x1
- up ramp x4 1-2 ft with mod A due to patient attempting to return to weight-bearing through elbows and roll toward left side

(97112) Neuromuscular Re-education to improve movement, balance, coordination, kinesthetic sense, proprioception, and posture - Direct Treatment Time: 25 minutes

Trunk ROM

*Facilitation of right side-sitting due to preference for left weight-shift in sitting. Patient maintains for approximately 3-5 sec before resisting x2
NPT*Trunk rotation AROM in sitting reaching for toys placed at his sides with prompting to reach across midline. Maintained for 20-30 sec to each side
* Trunk rotation AROM in supported standing at mat table to reach for toy to each side x5,CGA to maintain balance with 1 UE support on support surface (improvement from previous session)

Strengthening:

NPT*Quadruped over therapist's leg due to strong preference to attempt returning to sitting: blocking at BLE to maintain. Pt reaching with each UE (increased RUE reaching compared to LUE due to preference for left weight shift). Maintained for 20-30 sec x3
* Quadruped with mod A at BLE: patient maintaining for approximately 5-10 x3 sec before resisting and returning to prone
*Maintaining squat while engaged in play with min-mod A for 15-20 sec x4

Balance:

*Supported standing 30-60 seconds at support surface: increased BLE ER and increased BOS demonstrated. PT facilitating lateral weight-shifting to each side as well as reaching overhead and reaching with trunk rotation
NPT*Standing with 1 UE support on mat table while reaching to spin toy on window: CGA-min A to maintain with only 1 UE support for 30 sec x1
*Supported standing mid-floor with min A and intermittent CGA for approximately 10-15 sec x3
NPT*Standing on air disc while engaged with toys at support surface, min-mod A x60 sec x1
NP*Tall kneeling on air disc while reaching forward and laterally to engage with beads x30 sec
*Tall kneeling at therapy bench with 1-2 UE support for 10-20 sec x2. Mod A to return to tall kneel due to preference to assume short kneel with wide BOS.
*Half kneeling at support surface with 1-2 UE support for 10-20 sec

with emphasis on L foot leading x4 due to RLE preference
NPT*Half kneeling over therapist's leg, mod A with emphasis on L
foot leading due to RLE preference. Maintained for 15-20 sec x2
NPT*Supported standing with min A at pelvis while reaching to
engage with squigz at window for 60 sec x2

NPT = not performed today.

Treatment Given

TREATMENT NOTE

Medical Necessity : Patient exhibits a need for skilled physical
therapy intervention to address deficits in strength, range of
motion, balance, coordination, mobility, and gross motor skills
Treatment performed by : Savanna Smith, PT, DPT , Savanna
Smith, PT, DPT

Diagnosis

1. Down syndrome, unspecified - Q90.9 (Primary)

Procedure Codes

- 97530 97530 - THERAPEUTIC ACTIVITIES. Units: 2.00.
Modifiers: GP
- 97112 97112 - NEUROMUSCULAR RE-ED. Units: 2.00. Modifiers:
GP

Follow Up

Patient will continue with treatment as previously prescribed,1
Week.

Appointment Provider: Kandis Dennis, PT



**Electronic signature of Kandis Dennis , DPT, 1138097 on
02/16/2026 at 06:36 PM CST**

Sign off status: Pending

**Houston Center for Pediatric Therapy
26865 Interstate 45 N
Suite 300
The Woodlands, TX 773801531
Tel: 281-292-4800
Fax: 281-292-9588**

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2Y 2M old Male, DOB: 10/24/2023

Account Number: 194924

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Home: 334-492-1546

Guarantor: In, Sokkay Insurance: Holista (Aetna-Therapy Providers) Payer ID: ATHAL

Referring: Michelle Suhendra

Appointment Facility: Houston Center for Pediatric Therapy

01/14/2026

Appointment Provider: Kandis Dennis, PT Resource: Smith, Savanna
Supervising Provider: Kandis Dennis, PT

Reason for Appointment

1. Physical Therapy

Examination

Physical Examination:

Recommended New Goals

The following goals will allow Milo to develop the necessary range of motion, strength, coordination, balance, and body awareness to advance toward independent age-appropriate mobility and independent age-appropriate gross motor skills.

1. Short Term Goal: Milo will maintain quadruped position for 30 sec with CGA or less 2/3 trials in order to demonstrate improved strength and prepare for crawling on hands and knees.

Baseline: Milo is able to assume quadruped position but only maintains briefly due to poor tolerance for position, requiring blocking to maintain due to resistance.

2. Short Term Goal: Milo will crawl forward 5 feet on hand and knees using cross-lateral pattern demonstrating proper lower extremity alignment 2 out of 3 trials.

Baseline: Limited progress toward crawling on hands and knees due to poor tolerance for quadruped position and preference to collapse into prone and roll toward LEFT side during attempts at facilitating creeping forward. Milo demonstrates a strong preference for scooting forward on his bottom using his RLE and LUE.

3. Short Term Goal: Milo will transition from sitting to tall kneeling at support surface demonstrating proper lower extremity alignment and BOS 3 out of 4 trials.

Baseline: Milo continues to perform with excessive BLE external rotation with preference to transition from ring sitting to tall kneeling, resulting in wide BOS. Patient requires min-mod A to perform from side-sitting due to resistance.

4. Short Term Goal: Milo will maintain tall kneeling without UE support with min A and appropriate alignment and BOS for 10 sec 3/4 trials in order to demonstrate improved core and hip strength for age-appropriate transitions.

Baseline: Tactile cues to perform with narrow BOS and neutral LE alignment due to preference for BLE external rotation and abduction. Once positioned, Milo maintains briefly at support surface but not

Progress Note: Kandis Dennis, PT 01/14/2026

mid-floor without UE support.

5. Short Term Goal: Milo will pull to stand via half kneeling with each foot leading 3/4 trials with min A in order to demonstrate symmetrical LE strength and prepare for standing and cruising.
Baseline: Strong preference to assume half kneel with RIGHT foot leading. Min-mod A to consistently transition into standing

6. Short Term Goal: Milo will perform trunk rotation while standing at support surface with 1 UE support to reach for toy to each side 3/4 trials without LOB with CGA.

Baseline: Milo is able to maintain standing with 2 UE support on support surface with SBA, consistently. Patient is not yet able to perform without UE support or support provided at trunk. Milo attempts to lower to the floor when therapist facilitates reaching from UE support or requires min-mod A at trunk to prevent LOB.

7. Short Term Goal: Milo will transition from standing to sitting using stable object for support demonstrating proper lower extremity alignment 3 out of 4 trials with CGA and without LOB.

Baseline: Milo has demonstrated good progress with controlled lowering but does not always maintained UE support on support surface. Milo also demonstrates tendency to extend LEFT knee, requiring occasional tactile cueing and blocking to maintain L foot contact with the floor.

8. Short Term Goal: Milo will sidestep 4 steps in each direction along stable object demonstrating proper lower extremity alignment 3 out of 4 trials with min A.

Baseline: mod-max A to perform

9. Short Term Goal: Milo will ambulate forward 5 ft with push-toy with min A 3/4 trials without LOB in order to prepare for ambulation.

Baseline: Mod A to perform 2 steps forward before attempting to lower to the ground.

10. Short Term Goal: Milo will maintain standing mid-floor without UE support with SBA for 5 sec 3/4 trials in order to demonstrate improved balance and stability in standing.

Baseline: Mod A

11. Short Term Goal: Milo will ambulate 10 feet with shoulder width base of support with 2 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: Milo does not attempt stepping with 2 hand-held support. Milo has demonstrated up to 2 steps with push-toy with mod A at trunk.

12. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Long Term Goals have been established, as discharge criteria for the dates requested for this episode of care:

1. Milo will pull to stand via half kneeling with each foot leading 3/4 trials IND in order to demonstrate symmetrical LE strength and prepare for standing and cruising.

Baseline: See short term goal

2. Milo will cruise along support surface 6 steps to each side 3/4 trials IND without LOB in order to demonstrate symmetrical weight-bearing and prepare for IND stepping.

Baseline: See short term goal

3. Milo will ambulate 10 feet with shoulder width base of support with 1 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: See short term goal

4. Milo will perform squat to/from stand 3/4 trials with min A with symmetrical weight-bearing in order to safely retrieve objects from the floor.

Baseline: Mod A. Demonstrates increased tendency to extend left knee.

5. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Clinical Assessment Summary

Doctor's Name: Dr. Michelle Suhendra, M.D.

Phone: 832-539-7532

Fax: 832-336-3809

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Physical Examination

Physical Therapy:

Subjective

Time In: 03:02 pm

Time Out: 03:55 pm

Present for session: Patient transitioned well from mother in waiting area to treatment area with PT.

Patient participation: Patient participated well overall with intermittent resistance to therapeutic handling, especially when facilitating quadruped and stepping.

Parent reported: Patient received his bilateral SMOs yesterday. Patient has been tolerated them fairly well but yelled at mother when she put them on prior to PT session.

Home exercise program: Parents to continue HEP including activities from Position to Play for sitting and prone and cervical strengthening

exercises and tummy time. Quadruped with or without support of roll or support and reaching with UEs, tall kneeling supported on low cushion with assist to hold position, standing supported by stable object, half kneeling with assist as needed (emphasis on LLE leading due to preference for RLE).

Note:

Received SMOs via Align 01/13/26.

Evaluated for SMOs at Align on 12/09.

Objective

(97530) Therapeutic Activities to progressively improve functional performance. Direct Treatment Time: 30 minutes

Transitions:

*Moderate assist to assume quadruped position from prone and sitting due to poor tolerance for quadruped position.

* Quadruped to tall kneel with CGA x2

*Transitioned from sitting to tall kneeling at 16" surface multiple reps with SBA but with hip ER due to transitioning from ring-sitting.

Performed from side-sitting x2 from each side with min-mod A

*Transition from prone to sitting: preference for performing from his left side with tactile cues and blocking to perform from his right side

*Transition from sitting to prone: min-mod A to position BLE due to preference for scooting on bottom.

* Sit to/from stand on therapist's leg: tactile cues or blocking to maintain feet in contact with floor due to extending L knee. Min A to stand. Min-mod A to facilitate bilateral knee flexion to return to sitting. Mass practice

* Sit < > stand from cube chair with 2 HHA x6

* Pull to stand via half kneel: assumed half kneel with R foot leading multiple reps with SBA-CGA. Blocking and tactile cues to assume with LEFT foot leading. mod A to pull up with emphasis on L foot leading x5

NPT* Pull to sit from supine on floor mat x2: head lag demonstrated during first 50%

NPT* Straddling peanut ball while reaching overhead to pull squigz from mirror x1 min, stabilization provided at pelvis

NPT* Sitting on therapy ball with facilitation of supine to sit with mod A x3

NPT* Side-lying to sit on therapy ball x1 from each side with mod A

Mobility:

NPT* Rolling supine to prone: Rolling toward the left and right independently.

NPT* Rolling prone to supine: preference for rolling toward left side demonstrated, requiring blocking to perform toward right side x1

NPT* Commando crawling forward: 2 ft x1 with tactile cues.

Resistance demonstrated to additional attempts due to preference for scooting

* Cruising with 2 UE support at support surface: 4-6 steps to each side with min-mod A for foot placement due to performing with increased hip ABD and wide BOS.

NPT* Forward stepping with push-toy: 2-4 steps forward with increased trunk flexion toward push-toy. Performed x2 with mod A at pelvis

NPT* Scooting forward on bottom with L arm wrapped against trunk

with fabrifoam to promote using each LE due to strong preference to perform with weight-bearing through LUE and scooting with RLE. Performed 3 ft forward with max A for weight-shifting to each side x6
NPT* Scooting with blocking of LUE via 2 HHA to promote using BLE to scoot forward due to strong RLE preference
NPT* Commando crawling backward down ramp x3 with SBA
NPT* Commando crawling up ramp: attempted to facilitate but patient demonstrated resistance with preference to transition back to sitting
* Crawling on hands and knees: attempted to facilitate, but patient resisted
* Forward stepping with 2 HHA: up to 2 steps x3
* Forward stepping with mod A at pelvis: 4-6 steps, multiple reps. Up to 10 ft demonstrated x2.

(97112) Neuromuscular Re-education to improve movement, balance, coordination, kinesthetic sense, proprioception, and posture - Direct Treatment Time: 23 minutes

Trunk ROM

NPT*Facilitation of right side-sitting due to preference for left weight-shift in sitting. Patient maintains for approximately 3-5 sec before resisting x2

NPT*Trunk rotation AROM in sitting reaching for toys placed at his sides with prompting to reach across midline. Maintained for 20-30 sec to each side

* Trunk rotation AROM in supported standing at mat table to reach for toy to each side x3,CGA to maintain balance with 1 UE support on support surface (improvement from previous session)

Strengthening:

NPT*Quadruped over therapist's leg due to strong preference to attempt returning to sitting; blocking at BLE to maintain. Pt reaching with each UE (increased RUE reaching compared to LUE due to preference for left weight shift). Maintained for 20-30 sec x3

* Quadruped with mod A at BLE: patient maintaining for approximately 5-10 x2 sec before resisting and returning to prone

*Maintaining squat while engaged in play with min-mod A for 10-20 sec x4

Balance:

*Supported standing 30-60 seconds at support surface: improved LE alignment with B SMOs donned. Occasional excessive BLE ER demonstrated with tactile cues to correct. PT facilitating multi-directional weight-shifting

NPT*Standing with 1 UE support on mat table while reaching to spin toy on window: CGA-min A to maintain with only 1 UE support for 30 sec x1

*Supported standing mid-floor with min A and intermittent CGA for approximately 20-30 sec, multiple reps. With SBA, patient maintained for 1-2 sec

NPT*Standing on air disc while engaged with toys at support surface, min-mod A x60 sec x1

NP*Tailor sitting on air disc while reaching forward and laterally to engage with beads x30 sec

NPT*Tall kneeling at therapy bench with 1-2 UE support for 10-20

sec x2. Mod A to return to tall kneel due to preference to assume short kneel with wide BOS.

*Half kneeling at support surface with 1-2 UE support for 10-20 sec with emphasis on L foot leading x4 due to RLE preference

T*Half kneeling over therapist's leg, mod A with emphasis on L foot leading due to RLE preference. Maintained for 10 sec x2

NPT*Supported standing with min A at pelvis while reaching to engage with squigz at window for 60 sec x2

NPT = not performed today.

Treatment Given

TREATMENT NOTE

Medical Necessity : Patient exhibits a need for skilled physical therapy intervention to address deficits in strength, range of motion, balance, coordination, mobility, and gross motor skills
Treatment performed by : Savanna Smith, PT, DPT. Amber Zimmer, SPT present for observation.

Diagnosis

1. Down syndrome, unspecified - Q90.9 (Primary)

Procedure Codes

- 97530 97530 - THERAPEUTIC ACTIVITIES. Units: 2.00. Modifiers: GP
- 97112 97112 - NEUROMUSCULAR RE-ED. Units: 2.00. Modifiers: GP

Follow Up

Patient will continue with treatment as previously prescribed, 1 Week.

Appointment Provider: Kandis Dennis, PT



Electronic signature of Kandis Dennis , DPT, 1138097 on 02/16/2026 at 06:36 PM CST

Sign off status: Pending

**Houston Center for Pediatric Therapy
26865 Interstate 45 N
Suite 300
The Woodlands, TX 773801531**

IN, Milo DOB: 10/24/2023 (2 yo M) Acc No. 194924 DOS: 01/14/2026

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Progress Note: Kandis Dennis, PT 01/14/2026

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In, Milo

2Y 2M old Male, DOB: 10/24/2023

Account Number: 194924

27156 Allegretto Dr, Spring, TX-77386-4914

Home: 334-492-1546

Guarantor: In, Sokkay Insurance: Holista (Aetna-Therapy Providers) Payer ID: ATHAL

Referring: Michelle Suhendra

Appointment Facility: Houston Center for Pediatric Therapy

01/21/2026

Appointment Provider: Kandis Dennis, PT Resource: Smith, Savanna
Supervising Provider: Kandis Dennis, PT

Reason for Appointment

1. Physical Therapy

Examination

Physical Examination:

Recommended New Goals

The following goals will allow Milo to develop the necessary range of motion, strength, coordination, balance, and body awareness to advance toward independent age-appropriate mobility and independent age-appropriate gross motor skills.

1. Short Term Goal: Milo will maintain quadruped position for 30 sec with CGA or less 2/3 trials in order to demonstrate improved strength and prepare for crawling on hands and knees.

Baseline: Milo is able to assume quadruped position but only maintains briefly due to poor tolerance for position, requiring blocking to maintain due to resistance.

2. Short Term Goal: Milo will crawl forward 5 feet on hand and knees using cross-lateral pattern demonstrating proper lower extremity alignment 2 out of 3 trials.

Baseline: Limited progress toward crawling on hands and knees due to poor tolerance for quadruped position and preference to collapse into prone and roll toward LEFT side during attempts at facilitating creeping forward. Milo demonstrates a strong preference for scooting forward on his bottom using his RLE and LUE.

3. Short Term Goal: Milo will transition from sitting to tall kneeling at support surface demonstrating proper lower extremity alignment and BOS 3 out of 4 trials.

Baseline: Milo continues to perform with excessive BLE external rotation with preference to transition from ring sitting to tall kneeling, resulting in wide BOS. Patient requires min-mod A to perform from side-sitting due to resistance.

4. Short Term Goal: Milo will maintain tall kneeling without UE support with min A and appropriate alignment and BOS for 10 sec 3/4 trials in order to demonstrate improved core and hip strength for age-appropriate transitions.

Baseline: Tactile cues to perform with narrow BOS and neutral LE alignment due to preference for BLE external rotation and abduction. Once positioned, Milo maintains briefly at support surface but not

Progress Note: Kandis Dennis, PT 01/21/2026

mid-floor without UE support.

5. Short Term Goal: Milo will pull to stand via half kneeling with each foot leading 3/4 trials with min A in order to demonstrate symmetrical LE strength and prepare for standing and cruising.
Baseline: Strong preference to assume half kneel with RIGHT foot leading. Min-mod A to consistently transition into standing

6. Short Term Goal: Milo will perform trunk rotation while standing at support surface with 1 UE support to reach for toy to each side 3/4 trials without LOB with CGA.

Baseline: Milo is able to maintain standing with 2 UE support on support surface with SBA, consistently. Patient is not yet able to perform without UE support or support provided at trunk. Milo attempts to lower to the floor when therapist facilitates reaching from UE support or requires min-mod A at trunk to prevent LOB.

7. Short Term Goal: Milo will transition from standing to sitting using stable object for support demonstrating proper lower extremity alignment 3 out of 4 trials with CGA and without LOB.

Baseline: Milo has demonstrated good progress with controlled lowering but does not always maintained UE support on support surface. Milo also demonstrates tendency to extend LEFT knee, requiring occasional tactile cueing and blocking to maintain L foot contact with the floor.

8. Short Term Goal: Milo will sidestep 4 steps in each direction along stable object demonstrating proper lower extremity alignment 3 out of 4 trials with min A.

Baseline: mod-max A to perform

9. Short Term Goal: Milo will ambulate forward 5 ft with push-toy with min A 3/4 trials without LOB in order to prepare for ambulation.

Baseline: Mod A to perform 2 steps forward before attempting to lower to the ground.

10. Short Term Goal: Milo will maintain standing mid-floor without UE support with SBA for 5 sec 3/4 trials in order to demonstrate improved balance and stability in standing.

Baseline: Mod A

11. Short Term Goal: Milo will ambulate 10 feet with shoulder width base of support with 2 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: Milo does not attempt stepping with 2 hand-held support. Milo has demonstrated up to 2 steps with push-toy with mod A at trunk.

12. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Long Term Goals have been established, as discharge criteria for the dates requested for this episode of care:

1. Milo will pull to stand via half kneeling with each foot leading 3/4 trials IND in order to demonstrate symmetrical LE strength and prepare for standing and cruising.

Baseline: See short term goal

2. Milo will cruise along support surface 6 steps to each side 3/4 trials IND without LOB in order to demonstrate symmetrical weight-bearing and prepare for IND stepping.

Baseline: See short term goal

3. Milo will ambulate 10 feet with shoulder width base of support with 1 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: See short term goal

4. Milo will perform squat to/from stand 3/4 trials with min A with symmetrical weight-bearing in order to safely retrieve objects from the floor.

Baseline: Mod A. Demonstrates increased tendency to extend left knee.

5. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Clinical Assessment Summary

Doctor's Name: Dr. Michelle Suhendra, M.D.

Phone: 832-539-7532

Fax: 832-336-3809

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Physical Examination

Physical Therapy:

Subjective

Time In: 03:01 pm

Time Out: 03:55 pm

Present for session: Patient transitioned well from mother in waiting area to treatment area with PT.

Patient participation: Patient participated well overall with intermittent resistance to therapeutic handling, especially when facilitating quadruped. Improved attempts at cruising this session.

Parent reported: Patient has been tolerating B SMOs well and has been cruising more at home.

Home exercise program:

* quadruped

* half kneeling with assist as needed (emphasis on LLE leading due to

preference for RLE)

* assisted squat to stand

* assisted static standing with support at pelvis

Note:

Received SMOs via Align 01/13/26.

Evaluated for SMOs at Align on 12/09.

Objective

(97530) Therapeutic Activities to progressively improve functional performance. Direct Treatment Time: 30 minutes

Transitions:

*Moderate assist to assume quadruped position from prone and sitting due to poor tolerance for quadruped position.

* Quadruped to tall kneel with CGA x3

*Transitioned from sitting to tall kneeling at 16" surface multiple reps with SBA but with hip ER due to transitioning from ring-sitting. Performed from side-sitting x2 from each side with min-mod A

*Transition from prone to sitting: preference for performing from his left side with tactile cues and blocking to perform from his right side

*Transition from sitting to prone: min-mod A to position BLE due to preference for scooting on bottom.

* Sit to/from stand on therapist's leg: tactile cues or blocking to maintain feet in contact with floor due to extending L knee. Min A to stand. Min-mod A to facilitate bilateral knee flexion to return to sitting. Mass practice

NPT* Sit < > stand from cube chair with 2 HHA x6

* Pull to stand via half kneel: assumed half kneel with R foot leading multiple reps with SBA-CGA. Blocking and tactile cues to assume with LEFT foot leading. mod A to pull up with emphasis on L foot leading x5

* Stand to half kneel with emphasis on LLE as stance LE x5

NPT* Pull to sit from supine on floor mat x2: head lag demonstrated during first 50%

NPT* Straddling peanut ball while reaching overhead to pull squigz from mirror x1 min, stabilization provided at pelvis

NPT* Sitting on therapy ball with facilitation of supine to sit with mod A x3

NPT* Side-lying to sit on therapy ball x1 from each side with mod A

Mobility:

* Cruising with 2 UE support at support surface: 4-6 steps to each side with min-mod A for foot placement due to performing with increased hip ABD and wide BOS. Patient is able to perform with SBA 4-6 steps to each side but demonstrates increased hip ABD

* Crawling on hands and knees: patient performed ~1 ft forward before lowering to the floor mat

NPT* Forward stepping with 2 HHA: up to 2 steps x3

* Forward stepping with mod A at pelvis: 4-6 steps, multiple reps.

NPT* Rolling supine to prone: Rolling toward the left and right independently.

NPT* Rolling prone to supine: preference for rolling toward left side demonstrated, requiring blocking to perform toward right side x1

NPT* Commando crawling forward: 2 ft x1 with tactile cues.

Resistance demonstrated to additional attempts due to preference for scooting

NPT* Forward stepping with push-toy: 2-4 steps forward with increased trunk flexion toward push-toy. Performed x2 with mod A at pelvis

NPT* Scooting forward on bottom with L arm wrapped against trunk with fabrifoam to promote using each LE due to strong preference to perform with weight-bearing through LUE and scooting with RLE. Performed 3 ft forward with max A for weight-shifting to each side x6

NPT* Scooting with blocking of LUE via 2 HHA to promote using BLE to scoot forward due to strong RLE preference

NPT* Commando crawling backward down ramp x3 with SBA

NPT* Commando crawling up ramp: attempted to facilitate but patient demonstrated resistance with preference to transition back to sitting

(97112) Neuromuscular Re-education to improve movement, balance, coordination, kinesthetic sense, proprioception, and posture - Direct Treatment Time: 24 minutes

Trunk ROM

* Trunk rotation AROM in supported standing at mat table to reach for toy to each side x3, CGA to maintain balance with 1 UE support on support surface (improvement from previous session)

NPT* Facilitation of right side-sitting due to preference for left weight-shift in sitting. Patient maintains for approximately 3-5 sec before resisting x2

NPT* Trunk rotation AROM in sitting reaching for toys placed at his sides with prompting to reach across midline. Maintained for 20-30 sec to each side

Strengthening:

* Quadruped with mod A at BLE: patient maintaining for approximately 5-10 x2 sec before resisting and returning to prone
* Maintaining squat while engaged in play with min-mod A for 10-20 sec x4

NPT* Quadruped over therapist's leg due to strong preference to attempt returning to sitting: blocking at BLE to maintain. Pt reaching with each UE (increased RUE reaching compared to LUE due to preference for left weight shift). Maintained for 20-30 sec x3

Balance:

* Supported standing 30-60 seconds at support surface: improved LE alignment with B SMOs donned. Occasional excessive BLE ER demonstrated with tactile cues to correct. PT facilitating multi-directional weight-shifting

* Half kneeling at support surface with 1-2 UE support for 10-20 sec with emphasis on L foot leading x4 due to RLE preference

NPT* Half kneeling over therapist's leg, mod A with emphasis on L foot leading due to RLE preference. Maintained for 10 sec x2

NPT* Standing with 1 UE support on mat table while reaching to spin toy on window: CGA-min A to maintain with only 1 UE support for 30 sec x1

* Supported standing mid-floor with min A and intermittent CGA for approximately 20-30 sec, multiple reps. With SBA, patient maintained for 1-2 sec

NPT*Standing on air disc while engaged with toys at support surface, min-mod A x60 sec x1
NPT*Tailor sitting on air disc while reaching forward and laterally to engage with beads x30 sec
NPT*Tall kneeling at therapy bench with 1-2 UE support for 10-20 sec x2. Mod A to return to tall kneel due to preference to assume short kneel with wide BOS.
NPT*Supported standing with min A at pelvis while reaching to engage with squigz at window for 60 sec x2

NPT = not performed today.

Treatment Given

TREATMENT NOTE

Medical Necessity : Patient exhibits a need for skilled physical therapy intervention to address deficits in strength, range of motion, balance, coordination, mobility, and gross motor skills
Treatment performed by : Savanna Smith, PT, DPT. Amber Zimmer, SPT present for observation

Diagnosis

1. Down syndrome, unspecified - Q90.9 (Primary)

Procedure Codes

- 97530 97530 - THERAPEUTIC ACTIVITIES. Units: 2.00. Modifiers: GP
- 97112 97112 - NEUROMUSCULAR RE-ED. Units: 2.00. Modifiers: GP

Follow Up

Patient will continue with treatment as previously prescribed, 1 Week.

Appointment Provider: Kandis Dennis, PT



Electronic signature of Kandis Dennis , DPT, 1138097 on 02/16/2026 at 06:36 PM CST

Sign off status: Pending

**Houston Center for Pediatric Therapy
26865 Interstate 45 N
Suite 300**

IN, Milo DOB: 10/24/2023 (2 yo M) Acc No. 194924 DOS: 01/21/2026

The Woodlands, TX 773801531
Tel: 281-292-4800
Fax: 281-292-9588

Progress Note: Kandis Dennis, PT 01/21/2026

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

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IN, Milo DOB: 10/24/2023 (2 yo M) Acc No. 194924 DOS: 02/04/2026



In, Milo

2Y 3M old Male, DOB: 10/24/2023

Account Number: 194924

27156 Allegretto Dr, Spring, TX-77386-4914

Home: 334-492-1546

Guarantor: In, Sokkay Insurance: Holista (Aetna-Therapy Providers) Payer ID: ATHAL

Referring: Michelle Suhendra

Appointment Facility: Houston Center for Pediatric Therapy

02/04/2026

Appointment Provider: Kandis Dennis, PT Resource: Smith, Savanna
Supervising Provider: Kandis Dennis, PT

Reason for Appointment

1. Physical Therapy

Examination

Physical Examination:

Recommended New Goals

The following goals will allow Milo to develop the necessary range of motion, strength, coordination, balance, and body awareness to advance toward independent age-appropriate mobility and independent age-appropriate gross motor skills.

1. Short Term Goal: Milo will maintain quadruped position for 30 sec with CGA or less 2/3 trials in order to demonstrate improved strength and prepare for crawling on hands and knees.

Baseline: Milo is able to assume quadruped position but only maintains briefly due to poor tolerance for position, requiring blocking to maintain due to resistance.

2. Short Term Goal: Milo will crawl forward 5 feet on hand and knees using cross-lateral pattern demonstrating proper lower extremity alignment 2 out of 3 trials.

Baseline: Limited progress toward crawling on hands and knees due to poor tolerance for quadruped position and preference to collapse into prone and roll toward LEFT side during attempts at facilitating creeping forward. Milo demonstrates a strong preference for scooting forward on his bottom using his RLE and LUE.

3. Short Term Goal: Milo will transition from sitting to tall kneeling at support surface demonstrating proper lower extremity alignment and BOS 3 out of 4 trials.

Baseline: Milo continues to perform with excessive BLE external rotation with preference to transition from ring sitting to tall kneeling, resulting in wide BOS. Patient requires min-mod A to perform from side-sitting due to resistance.

4. Short Term Goal: Milo will maintain tall kneeling without UE support with min A and appropriate alignment and BOS for 10 sec 3/4 trials in order to demonstrate improved core and hip strength for age-appropriate transitions.

Baseline: Tactile cues to perform with narrow BOS and neutral LE alignment due to preference for BLE external rotation and abduction. Once positioned, Milo maintains briefly at support surface but not

Progress Note: Kandis Dennis, PT 02/04/2026

mid-floor without UE support.

5. Short Term Goal: Milo will pull to stand via half kneeling with each foot leading 3/4 trials with min A in order to demonstrate symmetrical LE strength and prepare for standing and cruising.
Baseline: Strong preference to assume half kneel with RIGHT foot leading. Min-mod A to consistently transition into standing

6. Short Term Goal: Milo will perform trunk rotation while standing at support surface with 1 UE support to reach for toy to each side 3/4 trials without LOB with CGA.

Baseline: Milo is able to maintain standing with 2 UE support on support surface with SBA, consistently. Patient is not yet able to perform without UE support or support provided at trunk. Milo attempts to lower to the floor when therapist facilitates reaching from UE support or requires min-mod A at trunk to prevent LOB.

7. Short Term Goal: Milo will transition from standing to sitting using stable object for support demonstrating proper lower extremity alignment 3 out of 4 trials with CGA and without LOB.

Baseline: Milo has demonstrated good progress with controlled lowering but does not always maintained UE support on support surface. Milo also demonstrates tendency to extend LEFT knee, requiring occasional tactile cueing and blocking to maintain L foot contact with the floor.

8. Short Term Goal: Milo will sidestep 4 steps in each direction along stable object demonstrating proper lower extremity alignment 3 out of 4 trials with min A.

Baseline: mod-max A to perform

9. Short Term Goal: Milo will ambulate forward 5 ft with push-toy with min A 3/4 trials without LOB in order to prepare for ambulation.

Baseline: Mod A to perform 2 steps forward before attempting to lower to the ground.

10. Short Term Goal: Milo will maintain standing mid-floor without UE support with SBA for 5 sec 3/4 trials in order to demonstrate improved balance and stability in standing.

Baseline: Mod A

11. Short Term Goal: Milo will ambulate 10 feet with shoulder width base of support with 2 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: Milo does not attempt stepping with 2 hand-held support. Milo has demonstrated up to 2 steps with push-toy with mod A at trunk.

12. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Long Term Goals have been established, as discharge criteria for the dates requested for this episode of care:

1. Milo will pull to stand via half kneeling with each foot leading 3/4 trials IND in order to demonstrate symmetrical LE strength and prepare for standing and cruising.

Baseline: See short term goal

2. Milo will cruise along support surface 6 steps to each side 3/4 trials IND without LOB in order to demonstrate symmetrical weight-bearing and prepare for IND stepping.

Baseline: See short term goal

3. Milo will ambulate 10 feet with shoulder width base of support with 1 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: See short term goal

4. Milo will perform squat to/from stand 3/4 trials with min A with symmetrical weight-bearing in order to safely retrieve objects from the floor.

Baseline: Mod A. Demonstrates increased tendency to extend left knee.

5. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Clinical Assessment Summary

Doctor's Name: Dr. Michelle Suhendra, M.D.

Phone: 832-539-7532

Fax: 832-336-3809

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Physical Examination

Physical Therapy:

Subjective

Time In: 03:03 pm

Time Out: 03:56 pm

Present for session: Patient transitioned well from mother in waiting area to treatment area with PT.

Patient participation: Patient participated well overall with intermittent resistance to therapeutic handling, especially when facilitating quadruped. Patient demonstrated occasional resistance to facilitation of ambulation.

Parent reported: Patient has been standing a lot at home. Patient had a bruise on his cheek due to LOB at home when trying to stand by himself.

Home exercise program:

- * quadruped
- * half kneeling with assist as needed (emphasis on LLE leading due to preference for RLE)
- * assisted squat to stand
- * assisted static standing with support at pelvis

Note:

Received SMOs via Align 01/13/26.

Evaluated for SMOs at Align on 12/09.

Objective

(97530) Therapeutic Activities to progressively improve functional performance. Direct Treatment Time: 30 minutes

Transitions:

*Moderate assist to assume quadruped position from prone and sitting due to poor tolerance for quadruped position.

NPT*Quadruped to tall kneel with CGA x3

NPT*Transitioned from sitting to tall kneeling at 16" surface multiple reps with SBA but with hip ER due to transitioning from ring-sitting. Performed from side-sitting x2 from each side with min-mod A

*Transitioned from sitting into half kneel at support surface with R foot leading multiple reps with SBA.

NPT*Transition from prone to sitting: preference for performing from his left side with tactile cues and blocking to perform from his right side

NPT*Transition from sitting to prone: min-mod A to position BLE due to preference for scooting on bottom.

*Sit to/from stand on therapist's leg: tactile cues or blocking to maintain feet in contact with floor due to extending L knee. Min A to stand and provide correct foot placement (B feet tend to outtoe). Min-mod A to facilitate bilateral knee flexion to return to sitting.

NPT* Sit < > stand from cube chair with 2 HHA x6

*Pull to stand via half kneel: assumed half kneel with R foot leading multiple reps with SBA-CGA. Blocking and tactile cues to assume with LEFT foot leading. mod A to pull up with emphasis on L foot leading x5

*Stand to half kneel on R/L with emphasis on LLE as anterior LE x5

NPT*Pull to sit from supine on floor mat x2: head lag demonstrated during first 50%

NPT*Straddling peanut ball while reaching overhead to pull squigz from mirror x1 min, stabilization provided at pelvis

NPT*Sitting on therapy ball with facilitation of supine to sit with mod A x3

NPT*Side-lying to sit on therapy ball x1 from each side with mod A

Mobility:

*Cruising with 2 UE support at support surface: 4-6 steps to each side with min-mod A for foot placement due to performing with increased hip ABD, outtoeing, and wide BOS. Patient is able to perform with SBA 4-6 steps to each side but demonstrates increased hip ABD.

NPT*Crawling on hands and knees: patient performed ~1 ft forward before lowering to the floor mat

*Forward stepping with min-mod A at pelvis: 4-6 steps, multiple reps. Increased assistance required over time due to increased resistance.

NPT* Forward stepping with 2 HHA: up to 2 steps x3

NPT* Rolling supine to prone: Rolling toward the left and right independently.
NPT* Rolling prone to supine: preference for rolling toward left side demonstrated, requiring blocking to perform toward right side x1
NPT*Commando crawling forward: 2 ft x1 with tactile cues.
Resistance demonstrated to additional attempts due to preference for scooting
NPT* Forward stepping with push-toy: 2-4 steps forward with increased trunk flexion toward push-toy. Performed x2 with mod A at pelvis
NPT*Scooting forward on bottom with L arm wrapped against trunk with fabrifoam to promote using each LE due to strong preference to perform with weight-bearing through LUE and scooting with RLE. Performed 3 ft forward with max A for weight-shifting to each side x6
NPT*Scooting with blocking of LUE via 2 HHA to promote using BLE to scoot forward due to strong RLE preference
NPT*Commando crawling backward down ramp x3 with SBA
NPT*Commando crawling up ramp: attempted to facilitate but patient demonstrated resistance with preference to transition back to sitting

(97112) Neuromuscular Re-education to improve movement, balance, coordination, kinesthetic sense, proprioception, and posture - Direct Treatment Time: 23 minutes

Trunk ROM

*Trunk rotation AROM in supported standing at mat table to reach for toy to each side x3 with SBA

NPT*Facilitation of right side-sitting due to preference for left weight-shift in sitting. Patient maintains for approximately 3-5 sec before resisting x2

NPT*Trunk rotation AROM in sitting reaching for toys placed at his sides with prompting to reach across midline. Maintained for 20-30 sec to each side

Strengthening:

*Quadruped with mod A at BLE: patient maintaining for approximately 3 sec x5 before resisting and returning to prone

*Maintaining squat while engaged in play with min-mod A for 10-20 sec x4

NPT*Quadruped over therapist's leg due to strong preference to attempt returning to sitting; blocking at BLE to maintain. Pt reaching with each UE (increased RUE reaching compared to LUE due to preference for left weight shift). Maintained for 20-30 sec x3

Balance:

*Supported static standing x1-2 min at support surface with Squigz: improved LE alignment with B SMOs donned. Occasional excessive BLE ER demonstrated with tactile cues to correct. Provided CGA/min A to maintain position.

*Half kneeling at support surface with 1-2 UE support for 10-20 sec with emphasis on L foot leading x5 due to RLE preference

NPT*Half kneeling over therapist's leg, mod A with emphasis on L foot leading due to RLE preference. Maintained for 10 sec x2

NPT*Standing with 1 UE support on mat table while reaching to spin toy on window: CGA-min A to maintain with only 1 UE support for 30

sec x1

*Supported standing mid-floor with min A and intermittent CGA for approximately 20-30 sec, multiple reps. With SBA, patient maintained for 5-10 sec when distracted by ball tower toy

NPT*Standing on air disc while engaged with toys at support surface, min-mod A x60 sec x1

NPT*Tall sitting on air disc while reaching forward and laterally to engage with beads x30 sec

NPT*Tall kneeling at therapy bench with 1-2 UE support for 10-20 sec x2. Mod A to return to tall kneel due to preference to assume short kneel with wide BOS.

NPT*Supported standing with min A at pelvis while reaching to engage with squigz at window for 60 sec x2

NPT = not performed today.

Treatment Given

TREATMENT NOTE

Medical Necessity : Patient exhibits a need for skilled physical therapy intervention to address deficits in strength, range of motion, balance, coordination, mobility, and gross motor skills
Treatment performed by : Savanna Smith, PT, DPT and Amber Zimmer, SPT, supervised by Savanna Smith, PT, DPT

Diagnosis

1. Down syndrome, unspecified - Q90.9 (Primary)

Procedure Codes

- 97530 97530 - THERAPEUTIC ACTIVITIES. Units: 2.00. Modifiers: GP
- 97112 97112 - NEUROMUSCULAR RE-ED. Units: 2.00. Modifiers: GP

Follow Up

Patient will continue with treatment as previously prescribed, 1 Week.

Appointment Provider: Kandis Dennis, PT



Electronic signature of Kandis Dennis , DPT, 1138097 on 02/16/2026 at 06:36 PM CST

Sign off status: Pending

IN, Milo DOB: 10/24/2023 (2 yo M) Acc No. 194924 DOS: 02/04/2026

Houston Center for Pediatric Therapy
26865 Interstate 45 N
Suite 300
The Woodlands, TX 773801531
Tel: 281-292-4800
Fax: 281-292-9588

Progress Note: Kandis Dennis, PT 02/04/2026

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IN, Milo DOB: 10/24/2023 (2 yo M) Acc No. 194924 DOS: 02/11/2026



In, Milo

2Y 3M old Male, DOB: 10/24/2023

Account Number: 194924

27156 Allegretto Dr, Spring, TX-77386-4914

Home: 334-492-1546

Guarantor: In, Sokkay Insurance: Holista (Aetna-Therapy Providers) Payer ID: ATHAL

Referring: Michelle Suhendra

Appointment Facility: Houston Center for Pediatric Therapy

02/11/2026

Appointment Provider: Kandis Dennis, PT Resource: Smith, Savanna
Supervising Provider: Kandis Dennis, PT

Reason for Appointment

1. Physical Therapy

Examination

Physical Examination:

Recommended New Goals

The following goals will allow Milo to develop the necessary range of motion, strength, coordination, balance, and body awareness to advance toward independent age-appropriate mobility and independent age-appropriate gross motor skills.

1. Short Term Goal: Milo will maintain quadruped position for 30 sec with CGA or less 2/3 trials in order to demonstrate improved strength and prepare for crawling on hands and knees.

Baseline: Milo is able to assume quadruped position but only maintains briefly due to poor tolerance for position, requiring blocking to maintain due to resistance.

2. Short Term Goal: Milo will crawl forward 5 feet on hand and knees using cross-lateral pattern demonstrating proper lower extremity alignment 2 out of 3 trials.

Baseline: Limited progress toward crawling on hands and knees due to poor tolerance for quadruped position and preference to collapse into prone and roll toward LEFT side during attempts at facilitating creeping forward. Milo demonstrates a strong preference for scooting forward on his bottom using his RLE and LUE.

3. Short Term Goal: Milo will transition from sitting to tall kneeling at support surface demonstrating proper lower extremity alignment and BOS 3 out of 4 trials.

Baseline: Milo continues to perform with excessive BLE external rotation with preference to transition from ring sitting to tall kneeling, resulting in wide BOS. Patient requires min-mod A to perform from side-sitting due to resistance.

4. Short Term Goal: Milo will maintain tall kneeling without UE support with min A and appropriate alignment and BOS for 10 sec 3/4 trials in order to demonstrate improved core and hip strength for age-appropriate transitions.

Baseline: Tactile cues to perform with narrow BOS and neutral LE alignment due to preference for BLE external rotation and abduction. Once positioned, Milo maintains briefly at support surface but not

Progress Note: Kandis Dennis, PT 02/11/2026

mid-floor without UE support.

5. Short Term Goal: Milo will pull to stand via half kneeling with each foot leading 3/4 trials with min A in order to demonstrate symmetrical LE strength and prepare for standing and cruising.
Baseline: Strong preference to assume half kneel with RIGHT foot leading. Min-mod A to consistently transition into standing

6. Short Term Goal: Milo will perform trunk rotation while standing at support surface with 1 UE support to reach for toy to each side 3/4 trials without LOB with CGA.

Baseline: Milo is able to maintain standing with 2 UE support on support surface with SBA, consistently. Patient is not yet able to perform without UE support or support provided at trunk. Milo attempts to lower to the floor when therapist facilitates reaching from UE support or requires min-mod A at trunk to prevent LOB.

7. Short Term Goal: Milo will transition from standing to sitting using stable object for support demonstrating proper lower extremity alignment 3 out of 4 trials with CGA and without LOB.

Baseline: Milo has demonstrated good progress with controlled lowering but does not always maintained UE support on support surface. Milo also demonstrates tendency to extend LEFT knee, requiring occasional tactile cueing and blocking to maintain L foot contact with the floor.

8. Short Term Goal: Milo will sidestep 4 steps in each direction along stable object demonstrating proper lower extremity alignment 3 out of 4 trials with min A.

Baseline: mod-max A to perform

9. Short Term Goal: Milo will ambulate forward 5 ft with push-toy with min A 3/4 trials without LOB in order to prepare for ambulation.

Baseline: Mod A to perform 2 steps forward before attempting to lower to the ground.

10. Short Term Goal: Milo will maintain standing mid-floor without UE support with SBA for 5 sec 3/4 trials in order to demonstrate improved balance and stability in standing.

Baseline: Mod A

11. Short Term Goal: Milo will ambulate 10 feet with shoulder width base of support with 2 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: Milo does not attempt stepping with 2 hand-held support. Milo has demonstrated up to 2 steps with push-toy with mod A at trunk.

12. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Long Term Goals have been established, as discharge criteria for the dates requested for this episode of care:

1. Milo will pull to stand via half kneeling with each foot leading 3/4 trials IND in order to demonstrate symmetrical LE strength and prepare for standing and cruising.

Baseline: See short term goal

2. Milo will cruise along support surface 6 steps to each side 3/4 trials IND without LOB in order to demonstrate symmetrical weight-bearing and prepare for IND stepping.

Baseline: See short term goal

3. Milo will ambulate 10 feet with shoulder width base of support with 1 hand-held support demonstrating proper lower extremity alignment 3 out of 4 trials.

Baseline: See short term goal

4. Milo will perform squat to/from stand 3/4 trials with min A with symmetrical weight-bearing in order to safely retrieve objects from the floor.

Baseline: Mod A. Demonstrates increased tendency to extend left knee.

5. With structured education and home program, family will demonstrate carryover and stabilization of new skills as demonstrated by daily participation with caregiver assist in home activity program with emphasis on range of motion, strengthening, and gross motor skill development.

Baseline: HEP updated at each session.

Clinical Assessment Summary

Doctor's Name: Dr. Michelle Suhendra, M.D.

Phone: 832-539-7532

Fax: 832-336-3809

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Physical Examination

Physical Therapy:

Subjective

Time In: 03:01 pm

Time Out: 03:55 pm

Present for session: Patient transitioned well from mother in waiting area to treatment area with PT.

Patient participation: Patient participated well overall with intermittent resistance to therapeutic handling. Patient demonstrated resistance to facilitation of ambulation.

Parent reported: no new changes.

Home exercise program:

* quadruped

* half kneeling with assist as needed (emphasis on LLE leading due to preference for RLE)

- * assisted squat to stand
- * assisted static standing with support at pelvis

Note:

Received SMOs via Align 01/13/26.

Evaluated for SMOs at Align on 12/09.

Objective

(97530) Therapeutic Activities to progressively improve functional performance. Direct Treatment Time: 31 minutes

Transitions:

*Moderate assist to assume quadruped position from prone and sitting due to poor tolerance for quadruped position.

*Sit to/from stand on therapist's leg at mirror with Squigz: tactile cues or blocking to maintain feet in contact with floor due to extending L knee. Min A to stand and provide correct foot placement (B feet tend to outtoe). Min-mod A to facilitate bilateral knee flexion to return to sitting.

*Transitioned from sitting into half kneel at support surface with R foot leading multiple reps with SBA x5.

*Noted patient performed pull to stand via half kneel without help from PT x1.

NPT*Quadruped to tall kneel with CGA x3

NPT*Transitioned from sitting to tall kneeling at 16" surface multiple reps with SBA but with hip ER due to transitioning from ring-sitting. Performed from side-sitting x2 from each side with min-mod A

NPT*Transition from prone to sitting: preference for performing from his left side with tactile cues and blocking to perform from his right side

NPT*Transition from sitting to prone: min-mod A to position BLE due to preference for scooting on bottom.

NPT* Sit < > stand from cube chair with 2 HHA x6

*NPT Pull to stand via half kneel: assumed half kneel with R foot leading multiple reps with SBA-CGA. Blocking and tactile cues to assume with LEFT foot leading. mod A to pull up with emphasis on L foot leading x5

*Stand to half kneel on R/L with emphasis on LLE as anterior LE x5

NPT*Pull to sit from supine on floor mat x2: head lag demonstrated during first 50%

NPT*Straddling peanut ball while reaching overhead to pull squigz from mirror x1 min, stabilization provided at pelvis

NPT*Sitting on therapy ball with facilitation of supine to sit with mod A x3

NPT*Side-lying to sit on therapy ball x1 from each side with mod A

Mobility:

*Cruising with 2 UE support at support surface: 4-6 steps to each side with min-mod A for foot placement due to performing with increased hip ABD, outtoeing, and wide BOS. Patient is able to perform with SBA 4-6 steps to each side but demonstrates increased hip ABD.

*Forward stepping with min-mod A at pelvis: 3-4 steps. Increased assistance required over time due to increased resistance.

NPT*Crawling on hands and knees: patient performed ~1 ft forward before lowering to the floor mat

NPT* Forward stepping with 2 HHA: up to 2 steps x3

NPT* Rolling supine to prone: Rolling toward the left and right

independently.

NPT* Rolling prone to supine: preference for rolling toward left side demonstrated, requiring blocking to perform toward right side x1

NPT*Commando crawling forward: 2 ft x1 with tactile cues.

Resistance demonstrated to additional attempts due to preference for scooting

NPT* Forward stepping with push-toy: 2-4 steps forward with increased trunk flexion toward push-toy. Performed x2 with mod A at pelvis

NPT*Scooting forward on bottom with L arm wrapped against trunk with fabrifoam to promote using each LE due to strong preference to perform with weight-bearing through LUE and scooting with RLE.

Performed 3 ft forward with max A for weight-shifting to each side x6

NPT*Scooting with blocking of LUE via 2 HHA to promote using BLE to scoot forward due to strong RLE preference

NPT*Commando crawling backward down ramp x3 with SBA

NPT*Commando crawling up ramp: attempted to facilitate but patient demonstrated resistance with preference to transition back to sitting

(97112) Neuromuscular Re-education to improve movement, balance, coordination, kinesthetic sense, proprioception, and posture - Direct Treatment Time: 23 minutes

Trunk ROM

*Seated trunk rotation + reaching with contralateral UE blocked (R rotation + L reaching, L rotation + R reaching) x7 each side. Patient needed mod-maxA for correctly reaching past midline.

NPT*Trunk rotation AROM in supported standing at mat table to reach for toy to each side x3 with SBA

NPT*Facilitation of right side-sitting due to preference for left weight-shift in sitting. Patient maintains for approximately 3-5 sec before resisting x2

NPT*Trunk rotation AROM in sitting reaching for toys placed at his sides with prompting to reach across midline. Maintained for 20-30 sec to each side

Strengthening:

*Maintaining squat while engaged in play with min-mod A for 10-20 sec x5

NPT* Quadruped with mod A at BLE: patient maintaining for approximately 3 sec x5 before resisting and returning to prone

NPT*Quadruped over therapist's leg due to strong preference to attempt returning to sitting: blocking at BLE to maintain. Pt reaching with each UE (increased RUE reaching compared to LUE due to preference for left weight shift). Maintained for 20-30 sec x3

Balance:

*Supported static standing x1-2 min at support surface with Squigz: improved LE alignment with B SMOs donned. Occasional excessive BLE ER demonstrated with tactile cues to correct. Provided CGA/min A to maintain position.

NPT*Half kneeling at support surface with 1-2 UE support for 10-20 sec with emphasis on L foot leading x5 due to RLE preference

NPT*Half kneeling over therapist's leg, mod A with emphasis on L foot leading due to RLE preference. Maintained for 10 sec x2

NPT*Standing with 1 UE support on mat table while reaching to spin toy on window: CGA-min A to maintain with only 1 UE support for 30 sec x1

NPT*Supported standing mid-floor with min A and intermittent CGA for approximately 20-30 sec, multiple reps. With SBA, patient maintained for 5-10 sec when distracted by ball tower toy

NPT*Standing on air disc while engaged with toys at support surface, min-mod A x60 sec x1

NPT*Tall sitting on air disc while reaching forward and laterally to engage with beads x30 sec

NPT*Tall kneeling at therapy bench with 1-2 UE support for 10-20 sec x2. Mod A to return to tall kneel due to preference to assume short kneel with wide BOS.

NPT*Supported standing with min A at pelvis while reaching to engage with squigz at window for 60 sec x2

NPT = not performed today.

Treatment Given

TREATMENT NOTE

Medical Necessity : Patient exhibits a need for skilled physical therapy intervention to address deficits in strength, range of motion, balance, coordination, mobility, and gross motor skills

Treatment performed by : Amber Zimmer, SPT, supervised by Savanna Smith, PT, DPT

Diagnosis

1. Down syndrome, unspecified - Q90.9 (Primary)

Procedure Codes

- 97530 97530 - THERAPEUTIC ACTIVITIES. Units: 2.00. Modifiers: GP
- 97112 97112 - NEUROMUSCULAR RE-ED. Units: 2.00. Modifiers: GP

Follow Up

Patient will continue with treatment as previously prescribed, 1 Week.

Appointment Provider: Kandis Dennis, PT



Electronic signature of Kandis Dennis , DPT, 1138097 on 02/16/2026 at 06:36 PM CST

Sign off status: Pending

IN, Milo DOB: 10/24/2023 (2 yo M) Acc No. 194924 DOS: 02/11/2026

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Progress Note: Kandis Dennis, PT 02/11/2026

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